Prior Authorization Request Form

Synthetic Medicare Advantage Plan - Utilization Management Department

Date submitted	11/02/2026
Request type	Standard (non-urgent), pre-service
Member name	Alex Q. Sample
Date of birth	03/15/1958
Member ID	SYN-MA-0001234
Requesting provider	Morgan Ellis, MD, Riverbend Family Medicine
Provider NPI	1000000000 (synthetic)
Requested service	MRI lumbar spine without contrast
CPT code	72148, 1 unit
Diagnosis	M54.16 Radiculopathy, lumbar region
Servicing facility	Lakeview Imaging Center (synthetic)

Attachments

Letter of medical necessity (page 2)

Office visit note, 10/28/2026 (pages 3-5)

Physical therapy discharge summary (pages 6-7)

Office visit note, 08/20/2026 (page 8)

Riverbend Family Medicine

November 2, 2026

To: Utilization Management, Synthetic Medicare Advantage Plan

Re: Alex Q. Sample, Member ID SYN-MA-0001234

Letter of Medical Necessity

I am requesting authorization for an MRI of the lumbar spine without contrast for my patient, Alex Q. Sample, who has lumbar radiculopathy.

The patient has failed conservative management and continues to have pain. Please see the attached records for details.

I believe this study is medically necessary for further evaluation.

Sincerely,

Morgan Ellis, MD

Riverbend Family Medicine

Office Visit Note

Riverbend Family Medicine. Date of service: 10/28/2026. Provider: Morgan Ellis, MD.

Patient: Alex Q. Sample. Age: 68.

Chief complaint

Low back pain radiating to the right leg.

History of present illness

The patient reports low back pain that began in the spring of 2026. Since early August 2026 the pain radiates from the right buttock down the lateral thigh and calf to the top of the right foot, with intermittent numbness of the right great toe.

The patient completed a course of physical therapy and continues ibuprofen. Current pain is 6 out of 10 and worse with prolonged sitting.

Past medical history

Hypertension. Hyperlipidemia.

Imaging history

Lumbar spine MRI without contrast on 06/18/2026 at Lakeview Imaging Center showed a mild L4-L5 disc bulge without significant canal stenosis. The report is not attached.

Red flag review

The patient denies bowel or bladder changes, saddle anesthesia, fever, unexplained weight loss, history of cancer, or recent trauma. No red flags identified.

Office Visit Note, continued (10/28/2026)

Medications

Lisinopril 10 mg daily. Atorvastatin 20 mg daily. Ibuprofen 600 mg up to three times daily with food.

Allergies

No known drug allergies.

Vital signs

Blood pressure 132/78. Heart rate 72. Temperature 98.4 F. BMI 27.1.

Physical examination

Tenderness of the right lumbar paraspinal muscles at L4-L5.

Straight-leg raise is positive on the right at 40 degrees and negative on the left.

Strength is 4+/5 for right great toe extension, unchanged since 08/20/2026, and 5/5 elsewhere in both lower extremities.

Light touch sensation is decreased over the top of the right foot.

Reflexes are 2+ and symmetric at the knees and ankles. Gait is mildly antalgic.

Office Visit Note, continued (10/28/2026)

Assessment

1. Lumbar radiculopathy in a right L5 distribution (M54.16), persistent despite physical therapy and NSAIDs.

2. Hypertension, controlled.

Plan

Order MRI lumbar spine without contrast.

Continue the home exercise program and ibuprofen as needed.

Follow up in 3 weeks.

Electronically signed: Morgan Ellis, MD, 10/28/2026

Physical Therapy Discharge Summary

Riverbend Physical Therapy (synthetic). Therapist: Jamie Cole, PT, DPT.

Patient: Alex Q. Sample. Referring provider: Morgan Ellis, MD.

Referral diagnosis: lumbar radiculopathy, right.

Episode of care

Dates of care: 08/24/2026 to 10/16/2026. Total visits: 12.

Interventions

Therapeutic exercise including core stabilization and directional preference exercises.

Neural mobilization (nerve glides) for the right lower extremity.

Manual therapy to the lumbar spine.

Home exercise program issued at the first visit and progressed through the episode.

Physical Therapy Discharge Summary, continued

Outcome measures

Measure	Initial (08/24/2026)	Discharge (10/16/2026)
Pain, numeric rating scale	7/10	6/10
Oswestry Disability Index	46%	42%
Right straight-leg raise	35 degrees	40 degrees

Assessment

The patient demonstrated minimal improvement in pain and function over the episode of care.

Radicular symptoms in the right lower extremity persist.

The minimal clinically important difference was not reached on the Oswestry Disability Index.

Discharge

Discharged because progress plateaued. The patient is independent with the home exercise program and was referred back to the physician.

Signed: Jamie Cole, PT, DPT, 10/16/2026

Office Visit Note

Riverbend Family Medicine. Date of service: 08/20/2026. Provider: Morgan Ellis, MD.

History of present illness

The patient strained the low back in May 2026 and was seen at urgent care. A lumbar MRI was obtained in June 2026 and the back pain later improved.

About two weeks ago new right leg pain began, radiating below the knee, with tingling in the right foot.

Examination

Straight-leg raise is positive on the right at 35 degrees.

Strength is 4+/5 for right great toe extension and 5/5 elsewhere. Reflexes are normal.

Assessment and plan

Right lumbar radiculopathy, new onset.

Referral to physical therapy, 2 visits per week for 6 to 8 weeks.

Ibuprofen 600 mg up to three times daily with food.

Will reassess after physical therapy.

Electronically signed: Morgan Ellis, MD, 08/20/2026